Clinical Care-Plan Case Study: Total Knee Replacement on Post-Operative Day 1 with Kidney Disease and Insulin-Treated Diabetes

ADN Med-Surg II | Individual take-home clinical care-plan case study | Open book

Your patient

You are the primary nurse for the patient in Orthopedics 5 West room 512-2 on the 7:00 to 15:00 shift on Tuesday, October 20, 2026 (hospital day 2, post-operative day 1). Open the patient in the student EHR (Student View, or the patient file from your instructor) and set the simulation clock to 0700. She had a right total knee replacement yesterday and she also has kidney disease, diabetes treated with insulin, high blood pressure and obesity. Work the way you would at the start of a real shift.

Parts 1 to 5 must be based on what was charted BEFORE 0700. Notes written after 0700 (the 0830 surgery note, 1030 hospitalist note, 1100 dietitian note and 1200 case management note) are printed on the sealed page in Part 6.

How this case study works

This packet follows the nursing process: Assessment, Diagnosis, Planning, Implementation and Evaluation. You will also explain the pathophysiology and pharmacology behind your patient's care. Expect to spend about 5 hours. It is a take-home, individual, open-book assignment: use your textbooks, drug guide, notes and the chart.

Questions ask you to explain WHY, not only WHAT. Short, specific answers that use this patient's data earn more points than general textbook answers. Real charts are imperfect: you will be asked to catch conflicts and to question orders before you act.

Part	Nursing process step	Estimated time
1	Assessment: Patient Data and Chart Review (Assessment)	60 min
2	Pathophysiology: The Disease Process (Assessment / Analysis of cues)	60 min
3	Pharmacology and Medication Safety (Assessment / Analysis (and safe implementation))	50 min
4	Nursing Diagnoses and Priorities (Diagnosis)	25 min
5	Planning and Implementation (Planning / Implementation)	50 min
6	Evaluation: What Happened and What Next? (Evaluation)	25 min
7	Clinical Judgment (Next-Generation NCLEX Style) (Evaluation / Clinical judgment)	30 min
	Estimated total	**5 h 0 min**

Scoring

- The packet is worth 100 points. A score of 78 or higher is passing.
- Each numbered item shows its point value. Points are given only for what is written in the box.

Writing so your pages can be scored from photos

- Write in dark pen or very dark pencil, print clearly, and stay inside the box. Write the answer in the box with the same number as the question.
- Write your name and date at the top of every page (the header prints a line for it).

- If you need more room, write 'continued on the back' and put the item number on the back. Do not squeeze writing between boxes.
- Do not use abbreviations that are not on the approved list. Spell out drug names.
- When you finish: photograph EVERY page, one page per photo, on a flat surface in good light with all four corners and the page number in view. Check that every photo is readable before you upload.

Part 6 is sealed

- Do not read the sealed page at the start of Part 6 until Parts 1 to 5 are finished. Your plan must be based on what you knew at 0700.

Honor statement: I completed this work myself, using only my textbooks, notes, drug references and the patient chart.

Signature: _____ Date: _____

PART 1: ASSESSMENT: PATIENT DATA AND CHART REVIEW

Nursing process step: Assessment | About 60 minutes | 26 points

Collect and organize the data you would have at 0700 on 10/20. Where the chart does not document something, write 'not documented.' Use R and L for laterality.

1.1 Basic Patient Information *(3 pts)*

Where to look in the EHR: Patient Summary and Storyboard

Room Number:		**Age:**	
Gender:		**Admission Date:**	
Allergies (and reactions):		**Height / Weight / BMI:**	
Chief Complaint:		**Primary Diagnosis:**	
Code Status:			

1.2 Medical Diagnoses and How They Affect Her Surgery and Recovery *(3 pts)*

List FOUR other medical diagnoses from this patient's chart. For each, explain how it affects her knee replacement recovery or the nursing care you give.

Where to look in the EHR: Chart Review > H&P; Encounters > Hospital problem list

Medical diagnosis	How it affects the primary diagnosis (knee replacement and recovery)

1.3 Patient History *(3 pts)*

Where to look in the EHR: H&P; ED Provider Note; Nursing Admission Note; Case Management consult order; PT evaluation

Medical History:

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

Surgical History:

Family History:

Social History:

Home Situation and Discharge Needs:

Story of the Knee Problem Before Admission:

1.4 Head-to-Toe Assessment at the Start of Your Shift (use R and L for laterality) *(4 pts)*

There is no 10/20 assessment yet. Use the most recent charted assessment (10/19 2000) and the night nurse's 0645 note. If the chart does not document a finding, write 'not documented.'

Where to look in the EHR: Flowsheets (last entry 10/19 2000); Nursing Progress Note 0645; device record

Neurological / mental status:

Cardiovascular: rhythm ____ heart sounds ____ cap refill ____ edema (where) ____

Respiratory: breath sounds ____ effort ____ O2 ____ incentive spirometer ____

Skin and incision (dressing, drainage, staples, Braden):

Right leg neurovascular and mobility: color/warmth ____ pulses ____ sensation ____ toe/ankle movement ____ calf ____ activity ____

Pain (location, score, last PRN doses):

GI and GU (bowel sounds, nausea, last bowel movement, voiding, I&O 0300-0700):

IV / Lines / Devices / Safety measures:

1.5 Vital Sign Trend *(2 pts)*

Copy the vital signs at each time. Circle every abnormal value. Then describe the trend in one sentence.

Where to look in the EHR: Flowsheets > Vital Signs (or the Vitals graph)

Vital sign	10/19 1200	10/19 1600	10/19 2000	10/20 0000	10/20 0400

Temperature					
Heart rate					
Blood pressure					
Respiratory rate					
SpO2 (%)					
Pain (0-10)					

Trend in one sentence:

1.6 Laboratory Data *(4 pts)*

Choose the 8 laboratory results most important for this patient today. For each give the 10/20 value with the normal range, the trend since admission (10/19), and explain why it matters for THIS patient.

Where to look in the EHR: Lab Results

Lab	Result / normal range / trend	Why is this lab important for this patient?

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

1.7 Diagnostics and Surgical Facts *(2 pts)*

Where to look in the EHR: Chart Review > Imaging, Cardiology, Operative note, Procedure note

X-ray right knee (result):

EKG (result):

Operation and anesthesia (procedure, anesthesia type, blood loss, tourniquet time, drain, dressing):

Nerve block (type, drug, amount, expected numbness and how long):

1.8 Chart Reconciliation *(5 pts)*

Real charts contain conflicts, duplicates and gaps. Find FIVE places where this chart (as of 0700) is conflicting, impossible, or needs clarification before you act. Say where you found each and who you would clarify with and what you would ask.

Where to look in the EHR: Orders, MAR, Devices, Chart Review > Notes (H&P, consults, nursing notes, PT, OT), I&O

What is wrong or unclear (and where)	Who you would clarify with and what you would ask

PART 2: PATHOPHYSIOLOGY: THE DISEASE PROCESS

Nursing process step: Assessment / Analysis of cues | About 60 minutes | 20 points

Explain WHY this is happening. Use your own words (plain language) and tie each idea to this patient.

2.1 Pathophysiology of Knee Osteoarthritis and Why She Needed a Knee Replacement *(5 pts)*

In plain language and your own words, explain the pathophysiology of this patient's primary diagnosis: why the knee joint wore out, why it hurts the way it does, why conservative treatment failed, what the operation changed, and what her body does in response in the first days.

2.2 Cause-and-Effect Chain *(3 pts)*

Fill in each box (one or two lines) to show how events in this patient lead from the worn-out knee to the problems you will prevent after surgery.

1. Why did her knee wear out? (factors in THIS patient)	
▼	
2. Why was surgery needed?	
▼	
3. What did the operation do to the tissues and blood vessels?	
▼	
4. What is the body's response (swelling, pain, temperature, blood count,	

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

glucose)?	

▼

5. What happens if she does not move and breathe deeply?	

▼

6. How do the nerve block, enoxaparin, ice, PT and CPM interrupt the chain?	

2.3 How Do You Know? Findings and Mechanisms *(4 pts)*

List SIX findings from this patient's assessment, vital signs, labs or I&O that show how her body is responding to surgery. Beside each, explain the mechanism that causes it.

Where to look in the EHR: Notes, Flowsheets, Lab Results, I&O

Finding in THIS patient (with value)	Why it happens (mechanism)

2.4a Diabetes and Surgery *(2 pts)*

Her A1c is 8.0% and her glucose was 181 and 193. She takes glargine at bedtime and a lispro correction scale, and she eats only part of her meals. Explain (1) why her glucose is high on POD 1, (2) why that matters for the new knee, and (3) why insulin is riskier for HER than for a patient with normal kidneys.

Where to look in the EHR: Lab Results; Orders; MAR

2.4b Kidney Disease and Surgery *(2 pts)*

She has CKD stage 3 (creatinine 1.62, eGFR 34 today). Explain TWO ways her kidneys change her risks or care after knee surgery, using specific orders, drugs or labs from her chart.

2.5 Treatment *(2 pts)*

What are we doing to treat this diagnosis after surgery? Include medical treatments AND nursing monitoring.

2.6 Hemoglobin Trend *(2 pts)*

Her hemoglobin went from 11.5 to 9.9 (hematocrit 35.0 to 29.6). Is this drop expected? State the ordered notification and transfusion thresholds, explain why she tolerates anemia less well than some patients, and list what you would watch for during the shift.

Where to look in the EHR: Lab Results > Hemoglobin; Orders (transfusion threshold)

PART 3: PHARMACOLOGY AND MEDICATION SAFETY

Nursing process step: Assessment / Analysis (and safe implementation) | About 50 minutes | 14 points

Use your drug guide and the MAR (and the drug guide link inside the EHR). Part of your job is to question orders before you give them.

3.1 Medication List *(4 pts)*

From the MAR and orders, choose the SIX medications that matter most to you during the 0700-1500 shift. List brand and generic names.

Where to look in the EHR: MAR (click the medication name for the drug guide)

Medication (brand and generic)	Drug class and mechanism (10 words or fewer)	Why is THIS patient taking it?	Key nursing check / hold parameter

3.2a Opioid Orders in a Patient with Kidney Disease *(2 pts)*

Hydromorphone 0.5 mg IV (q3h PRN for pain 7-10) and oxycodone 5 mg PO (q4h PRN for pain 4-6) are ordered. Overnight she received hydromorphone at 1915 and 0330 and oxycodone at 0015; her charted pain scores were 5 and 6. Her pain is 5/10 now and PT is coming. What is wrong or unsafe in the record, what will you give and when, and how does her kidney function change opioid safety?

Where to look in the EHR: MAR (PRN section), Orders, Flowsheet pain scores, Nursing notes

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

3.2b The Obvious Action Is Wrong: Celecoxib *(2 pts)*

The H&P says acetaminophen AND celecoxib were started as part of multimodal analgesia, and the oxycodone order says to wean to 'acetaminophen and celecoxib.' There is no celecoxib on the MAR. At 0730 a coworker says, 'Just have the surgeon add the celecoxib 200 mg daily so she is on the full plan.' Decide what you will do, and explain every safety reason using HER chart.

Where to look in the EHR: H&P plan; oxycodone order text; MAR; Orders (renal dosing review); Allergies

3.2c Insulin When Intake Is Changing *(2 pts)*

It is 0730. Her 0500 lab glucose was 193. Breakfast is arriving and she has been eating about half to three-quarters of her meals. Lispro correction is due now, and glargine 20 units is due at 2100 tonight. What do you check and do before the lispro, why is hypoglycemia a bigger risk for her, how will you treat a low glucose on her renal diet, and what will you do before tonight's glargine?

Where to look in the EHR: MAR, Orders (glargine, lispro, POC glucose), Labs, Dietitian orders

3.3 Math From the Chart *(4 pts)*

Show your work in the box. Include units.

Where to look in the EHR: Nerve block procedure note, Orders/MAR, Labs, Patient summary, I&O

a) The adductor canal block used 20 mL of ropivacaine 0.5%. How many mg were given? The maximum is 3 mg/kg: is the dose below the maximum for her 88 kg? (what percent of the maximum?)

b) Enoxaparin 40 mg daily is ordered (30 mg daily if CrCl is below 30 mL/min). Female Cockcroft-Gault: CrCl = (140 - age) x weight / (72 x creatinine) x 0.85. Use today's creatinine. Ideal body weight (female) = 45.5 + 2.3 x (inches over 60); adjusted weight = IBW + 0.4 x (actual - IBW). Height is 160 cm (2.54 cm = 1 inch). Calculate CrCl with her ACTUAL weight and with her ADJUSTED weight, then state if the ordered dose is acceptable and what you will do.

c) The glargine order says 20 units 'reduced 20% while NPO.' If 20 units is the already-reduced dose, what was her home dose? If 20 units is her home dose (as the H&P lists), what would a 20% reduction be?

d) Using the I&O record and her weight, find the urine output in mL/kg/hour for 10/19 1100-1500 and for 10/20 0300-0700, and her 24-hour net fluid balance from 10/19 0700 to 10/20 0700. Is each urine output adequate (goal at least 0.5 mL/kg/hour)?

PART 4: NURSING DIAGNOSES AND PRIORITIES

Nursing process step: Diagnosis | About 25 minutes | 8 points

Use NANDA-I diagnoses. Write each as problem, 'related to' (cause you can treat), and 'as evidenced by' (specific data). Do not use a medical diagnosis as the 'related to.'

4.1 Prioritized Nursing Diagnoses *(6 pts)*

Write FOUR nursing diagnoses in priority order (1 = highest).

#	NANDA-I nursing diagnosis	Related to (etiology)	As evidenced by (at least 2 specific chart findings)

4.2 Why This Order? *(2 pts)*

Explain why your #1 diagnosis is first (use ABCs, Maslow, or acuity). Name one risk diagnosis you ranked lower that could become the top priority and the finding that would move it up.

PART 5: PLANNING AND IMPLEMENTATION

Nursing process step: Planning / Implementation | About 50 minutes | 14 points

Plan the care for the 0700-1500 shift on POD 1. SMART goal = Specific, Measurable, Achievable, Relevant, Time-bound, written about the PATIENT (not the nurse).

5.1a Plan for Priority Diagnosis #1: _____ *(2.5 pts)*

I = independent, C = collaborative, D = delegated.

SMART goal for this shift	Nursing interventions (at least 4: what, how often)	Rationale	I / C / D

5.1b Plan for Priority Diagnosis #2: _____ *(2.5 pts)*

SMART goal for this shift	Nursing interventions (at least 4: what, how often)	Rationale	I / C / D

5.2 Time-Sequenced Plan for the Shift *(3 pts)*

For each hour, list your priority actions IN ORDER. Include medications (and which you will question), assessments, mobility and therapy, teaching, and communication, and say why at that time.

Where to look in the EHR: MAR, Orders, Brain/Worklist, Secure Chat, Notes

Time	Priority actions in order	Why at this time
0700		
0800		
0900		
1000		
1100		
1200		
1300		
1400		

5.3 Calling the Provider (SBAR) *(2 pts)*

At 0730 you finish reviewing the chart. The plan mentions celecoxib but it is not ordered, the LR is still recorded as infusing after the order ended, the IV hydromorphone ends at 0900, and enoxaparin 40 mg is ordered with eGFR 34 and no documented dose check. You call the surgeon (Dr. Alvarez) or the hospitalist (Dr. Patel). Write what you would say.

Where to look in the EHR: MAR; Orders; Notes

S: Situation

B: Background

A: Assessment

R: Recommendation / request

5.4 Delegation *(1 pt)*

Mark the LOWEST level of staff who may be assigned each task for this patient.

	RN only	LPN/LVN	UAP/PCT
Walk her to the bathroom with the walker and gait belt after the RN has checked her blood pressure and found no dizziness	☐	☐	☐
Compare the pulses, color, warmth, sensation and movement of both feet and decide if the change is concerning	☐	☐	☐
Give the 0900 oral lisinopril after the RN has reviewed the blood pressure, potassium and creatinine	☐	☐	☐
Teach enoxaparin self-injection with teach-back for discharge	☐	☐	☐

5.5 Discharge Planning (Home vs Skilled Nursing Facility) and Teaching *(3 pts)*

The nursing handoff says discharge home on POD 2. Use the chart as it stood at 0700 to answer both parts.

Where to look in the EHR: Case management order, PT and OT evaluations, H&P social history, Nursing notes

a) What does the chart say about her home and support, what does PT say is needed for home vs a skilled nursing facility, what information is still MISSING, and what would you recommend right now (and who would you ask)?

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

b) Name THREE teaching topics she needs before discharge. For each say how you will teach it (adapt to her) and how you will confirm learning.

SEALED PAGE. Read only after Parts 1 to 5 are complete.

Chart notes written after 0700 on 10/20 (from the EHR), followed by the INSTRUCTOR ADDENDUM for the 0700-1500 shift. The addendum is NOT in the EHR; use these events as if you had charted them.

10/20 08:30 Surgery Progress Note

Hospital Day 2 | POD 1
Interval events / subjective
Knee aching 4-5/10 at rest and 6/10 with therapy after the block wore off overnight. Slept poorly. Passed gas but no bowel movement. Wants to go home soon.
- X-ray right knee, AP and lateral, post-operative: Satisfactory total knee arthroplasty.

Objective
- Vitals (24 h): Tmax 99.5 F; HR 74-87; BP 136-150/80-85; RR 15-17; SpO2 94-96%; pain 3-6/10
- Current: T 99.5 F, HR 80, BP 142/80, RR 17, SpO2 95% on room air, pain 5/10
- I&O (24 h): in 2778 mL / out 1743 mL
- Diet: Renal, consistent carbohydrate diet; Activity: Weight bearing as tolerated right leg with rolling walker, up with assistance

Exam
- General: Awake and alert, in no acute distress.
- Cardiovascular: Normal sinus rhythm; S1 S2 regular, no murmur. Edema: Right knee and lower leg swollen with ecchymosis (expected); calf soft, non-tender.
- Respiratory: Clear, diminished at bases; unlabored, even.
- Abdomen: Soft, non-tender, non-distended. Bowel sounds: active in all four quadrants.
- Neurologic: Alert and oriented x4.
- Skin: Warm, dry; right knee skin intact without rash, abrasions or lesions; surgical site marked.

Labs
- Hemoglobin 9.9 (L) (down from 11.5); WBC 10.4 (up from 7.0); Platelets 199 (down from 240); Sodium 137 (stable from 139); Potassium 4.7 (stable from 4.7); Creatinine 1.62 (H) (up from 1.51); Glucose 193 (H) (up from 181); INR 1.0

Medications
- Scheduled: acetaminophen 1,000 mg Oral; insulin glargine 20 units Subcutaneous; insulin lispro correction scale Low-dose correction scale Subcutaneous; lisinopril 20 mg Oral; atorvastatin 40 mg Oral; pantoprazole 40 mg Oral; levothyroxine 75 mcg Oral; enoxaparin 40 mg Subcutaneous; senna-docusate 2 tablets Oral
- PRN: hydromorphone; oxycodone; ondansetron; polyethylene glycol powder

Assessment and plan
71-year-old female, hospital day 2 | POD 1.
Primary osteoarthritis of the right knee
- Recovering as expected. Hgb about 2 g/dL below baseline without symptoms; temperature up to 99.6 F is common after surgery; incision clean; no signs of DVT, infection, or retention.
- IV morphine stopped; oxycodone only for pain above 4 before therapy; continue acetaminophen and celecoxib.
- PT twice a day: walking, stairs practice, ROM goals 0-90 degrees; CPM per order; ice and elevation.
- Daily CBC/BMP; transfuse only for Hgb below 7-8 or symptoms.
- Continue enoxaparin and SCDs; bowel regimen (senna-docusate scheduled, Miralax PRN).
- Case management: home health vs SNF, walker and raised toilet seat ordered.

Chronic kidney disease stage 3
- Baseline Cr about 1.8; renally dose all medications and avoid nephrotoxins.

10/20 10:30 Hospitalist Co-management Progress Note

Interval
- No acute medical events; surgical team managing the primary problem.

Assessment and plan
Chronic kidney disease stage 3

- Baseline Cr about 1.8; renally dose all medications and avoid nephrotoxins.
- Monitor BMP daily; strict I&O.

Obesity
- Weight-appropriate DVT prophylaxis dosing; early mobilization; skin checks every shift.

Type 2 diabetes mellitus
- Basal glargine with ACHS correction lispro; goal glucose 140-180 mg/dL.
- Metformin held while inpatient.
- Hypoglycemia protocol in place.

Hypertension
- Continue home antihypertensive; BP goal below 140/90.

Hyperlipidemia
- Continue statin.

Gastroesophageal reflux disease
- Continue pantoprazole (IV while NPO).

Hypothyroidism
- Continue levothyroxine when taking PO (may hold a few days if NPO); TSH on admission.

10/20 11:00 Registered Dietitian Nutrition Assessment

Assessment
- Ht 160 cm, Wt 88 kg, BMI 34.4.
- Current diet: Renal, consistent carbohydrate diet. Intake: fair, about 50-75% of meals.
- Pertinent labs: Sodium 137 (stable from 139); Potassium 4.7 (stable from 4.7); Glucose 193 (H) (up from 181); Creatinine 1.62 (H) (up from 1.51); Albumin 4.1; Phosphorus 4.1.

Estimated needs
- Energy 2200-2650 kcal/day; protein 88-106 g/day; fluid per medical team.

Recommendations
- Continue Renal, consistent carbohydrate diet; encourage protein at each meal; oral supplement if intake stays below 50%.
- Consistent carbohydrate teaching; Renal diet teaching (sodium, potassium, phosphorus); follow up in 3-4 days.

10/20 12:00 Case Management Initial Assessment

Social situation
- Lives with spouse/partner. Insurance: Medicare with supplemental plan. Transportation: family/self.
- Prior function: independent with a cane for 2 years.

Barriers
- uses independent with a cane for 2 years at baseline

Anticipated discharge plan
- Home with home health or outpatient PT, rolling walker and a family caregiver (skilled nursing facility only if needed).
- Estimated discharge: post-operative day 2 (hospital day 3) once pain is controlled on oral medication, walking 100+ feet with a walker, stairs completed, voiding and Hgb stable.

Plan
- Follow daily for medical readiness, therapy recommendations and equipment/home health needs.

INSTRUCTOR ADDENDUM (not in the EHR) 0730-0900 Morning

- Point-of-care glucose 214 at 0730. She ate about half of her breakfast. The low-dose lispro correction was given with the meal. At 0900 BP 144/82; her scheduled morning medications were given as ordered. Oxycodone 5 mg PO was given at 0945 for pain 5/10 before therapy.

1015 PT session

- Walked 100 feet with the rolling walker and assist of one. Knee flexion 78 degrees. Stair trial: 4 steps with the rail needed moderate assistance from the therapist plus a second person; pain rose to 7/10 and she stopped. She says her bedroom is upstairs, that she is afraid of the stairs, and that her spouse uses a cane and has a bad back and cannot lift or steady her. Her daughter lives 20 minutes away and works days.

1130 Before lunch

- She tells the nurse she feels shaky, sweaty and dizzy. Glucose 62. She tried to stand from the chair by herself to go to the bathroom and her right knee buckled; the aide caught her and lowered her to the chair. No fall to the floor, no head strike, dressing intact.

1145-1300 After treatment

- Juice given per protocol; glucose 91 at 1145 and she ate 75% of lunch; the lispro correction for lunch was not given. At 1300 she is calm and says she wants to go home tomorrow. R foot warm, pulses palpable, capillary refill under 3 seconds, toes move, calf soft and non-tender, dressing dry. No bowel movement yet this admission that is charted.

Time	HR	BP	RR	SpO2	Temp	Pain	Other
0400 (chart)	80	142/80	17	95% RA	99.5 F	5/10	from the flowsheet
0800	84	144/82	16	95% RA	99.4 F	5/10	glucose 214 (0730)
1015	96	130/76	18	94% RA	-	7/10	during stairs
1130	98	108/64	18	96% RA	98.9 F	4/10	glucose 62; shaky, diaphoretic
1145	90	118/70	16	96% RA	-	4/10	glucose 91 after juice
1300	86	126/74	16	95% RA	99.6 F	5/10	ate 75% of lunch

Coming up: acetaminophen 1,000 mg is due at 1400; lispro with supper 1630; glargine 20 units at 2100 (order: notify the provider before giving if glucose is below 100); enoxaparin 40 mg at 2145.

PART 6: EVALUATION: WHAT HAPPENED AND WHAT NEXT?

Nursing process step: Evaluation | About 25 minutes | 8 points

Open the sealed page. It shows the real chart notes written after 0700 and an addendum written by your instructor that describes what happened during your shift (the addendum is not in the EHR). Do not change your Part 1-5 answers.

6.1 Evaluating Your Goals *(3 pts)*

For each goal you wrote in 5.1a and 5.1b, use data from the sealed page to say whether it was met, partly met, or not met, and what you will change.

Goal (copy from Part 5)	Data from the sealed page that you used	Met / Partly / Not	Revised plan

6.2 New Problems and Priority Actions *(3 pts)*

From the sealed page, name the TWO most urgent new problems. For each give your priority nursing actions: what you do at the bedside, whom you notify, what you will do about tonight's glargine, and how the stair trial and the family situation change the discharge plan you wrote in 5.5.

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

6.3 Focused Nursing Note *(2 pts)*

Write a focused nursing note for the 1130 event (DAR, SOAP, or your school's format): data, action, and response, with the plan.

PART 7: CLINICAL JUDGMENT (NEXT-GENERATION NCLEX STYLE)

Nursing process step: Evaluation / Clinical judgment | About 30 minutes | 10 points

Answer by marking the boxes. These items use the same patient. Some scenarios are hypothetical and say so.

7.1 Select All That Apply *(2 pts)*

Which nursing actions are correct for this patient on post-operative day 1?

Select ALL that apply.

☐ Compare the color, warmth, cap refill, pulses, sensation and toe/ankle movement of the right foot with the left every 4 hours.

☐ Place a pillow under the right knee so it rests bent and comfortable.

☐ Apply the ice pack directly to the skin under the dressing for an hour because the nerve block is gone.

☐ Check blood pressure, potassium and creatinine before the lisinopril, and hold it and call if SBP is below 100.

☐ Give enoxaparin 40 mg without any checks because it is a standard prophylaxis dose.

☐ Review the platelet count, creatinine/CrCl and bleeding signs when giving enoxaparin, and report platelets below 50 or active bleeding.

☐ Offer ibuprofen from the unit stock for knee pain because it is an over-the-counter drug.

☐ Give oxycodone 30-60 minutes before PT and reassess pain and sedation afterward.

7.2 Trend Matrix: Expected or Report Now? *(2 pts)*

HYPOTHETICAL: on post-operative day 2 you find each of these trends compared with POD 1. Mark one box per row.

	Expected after surgery	Report now
Hemoglobin 9.9 to 9.5 g/dL, HR 82, BP 138/78, no dizziness, dressing dry	☐	☐
Creatinine 1.62 to 2.0 mg/dL (up 0.38), potassium 4.7 to 5.4 mmol/L, urine output about 20 mL/hour for 4 hours	☐	☐
New right calf tenderness and warmth, calf larger than the left, HR 104	☐	☐
Temperature 99.5 to 99.8 F, WBC 10.4 to 9.6, incision edges dry with bruising	☐	☐

7.3 Bowtie *(2 pts)*

HYPOTHETICAL post-operative day 1 afternoon: she reports pain 9/10 in the right calf that is not relieved by oxycodone. The calf is tense and swollen, the right foot is cool and pale with capillary refill of 5 seconds, she has tingling in the toes and severe pain when the nurse moves her toes. Pulses are faint. The CPM machine is running.

Actions to Take (select 2)	Condition the patient is most likely experiencing (select 1)	Parameters to Monitor (select 2)

Write one answer per numbered box, inside the box, in dark pen, neatly. Open book, individual work.

☐ Stop the CPM and call the surgeon immediately with SBAR ☐ Give another dose of oxycodone and recheck in one hour ☐ Raise the leg high on three pillows and add more ice ☐ Do a focused neurovascular check of both legs and stay with her ☐ Document the findings and recheck in four hours	☐ Compartment syndrome / acute neurovascular compromise ☐ Expected post-operative swelling ☐ Surgical site infection ☐ Hypoglycemia	☐ Capillary refill, color, temperature and pulses of the right foot ☐ Pain with passive toe movement and tightness of the calf ☐ Bowel sounds ☐ Blood glucose ☐ Urine color

7.4 Recognize Cues *(2 pts)*

Excerpt from a HYPOTHETICAL post-operative day 1 evening shift report. Which statements need follow-up NOW?

Draw a line under each statement that needs follow-up now. Then write the item numbers in the box below.

(1) Dressing clean, dry and intact; knee swollen with bruising; calf soft.

(2) Walked 80 feet with the walker and assist of one; pain 4/10 after oxycodone.

(3) The numbness over her inner knee and shin has not gone away 30 hours after the block, and she says her foot feels like it flaps when she walks.

(4) Glucose 152 before breakfast and 168 before supper; ate about 60% of her meals.

(5) She says she took two ibuprofen from her purse this afternoon for knee pain.

(6) Voided 350 mL twice this shift; no fullness over the bladder.

(7) No bowel movement since admission 2 days ago; she has taken oxycodone and says the senna 'upsets her stomach' so she has skipped it.

(8) Temperature 99.4 F, SpO2 95% on room air, incentive spirometer 1,500 mL.

Item numbers that need follow-up:

7.5 Priority Actions *(2 pts)*

HYPOTHETICAL: on post-operative day 2 she is walking with PT when she suddenly becomes short of breath with sharp chest pain, HR 126, RR 30, SpO2 86% on room air. Number the actions in the order you would do them.

Number the actions 1 (first) to 5 (last).

[___] Call the rapid response team (or surgeon) with SBAR and stay with her.
[___] Raise the head of the bed, apply oxygen and attach continuous pulse oximetry and a cardiac monitor.
[___] Stop the activity, help her to sit or lie down, and assess airway, breathing and circulation.
[___] Confirm IV access and gather the MAR (enoxaparin times), allergies and latest labs for the team.
[___] Document the event and update the family.